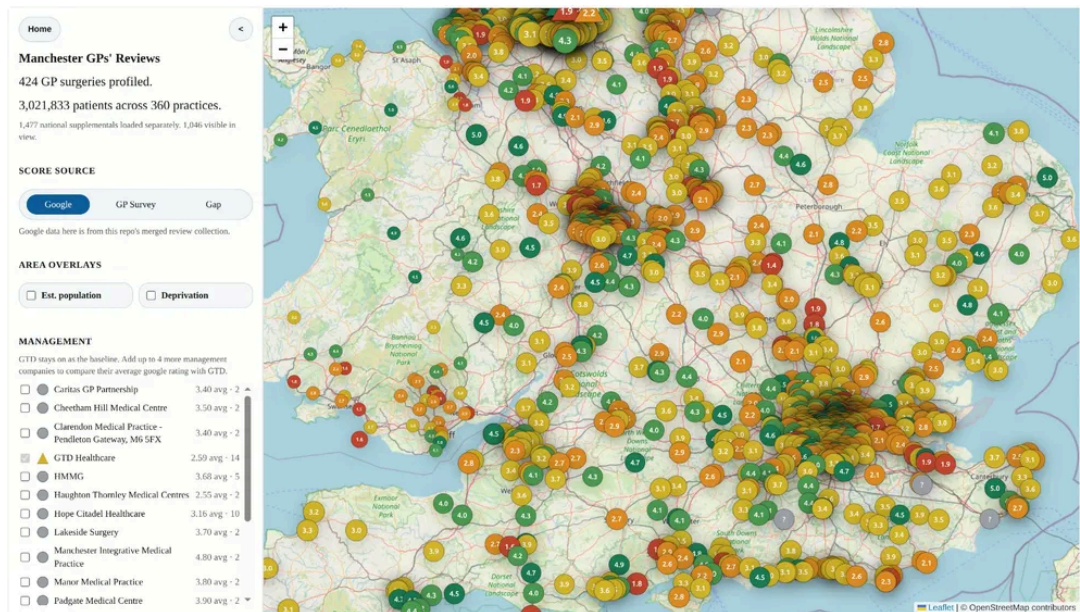


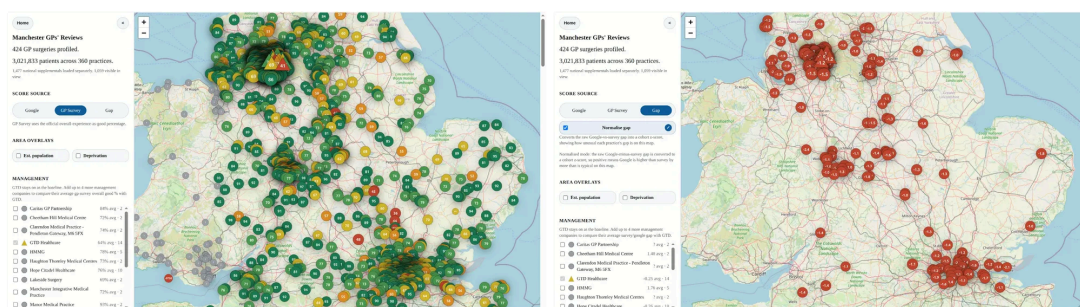
[← Process Another Thread](#)[View Original Thread](#)[Back to Home](#)**Bob Davies**

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About a fifth of the way



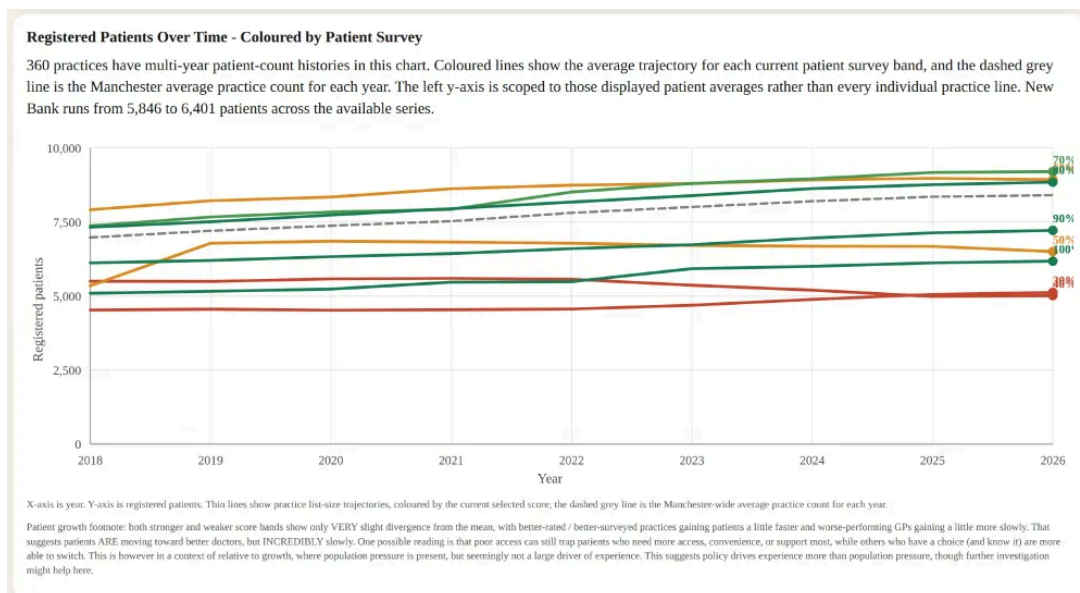
Compared to the patient survey. The last is national reviews-vs-survey gap. I don't have a deprivation layer for the entire map (rendering 7000 markers AND 100k+ deprivation polygons is slow, so I'll need to pre-render an overlay) but this looks so far like poorer/urban practices have larger gaps.



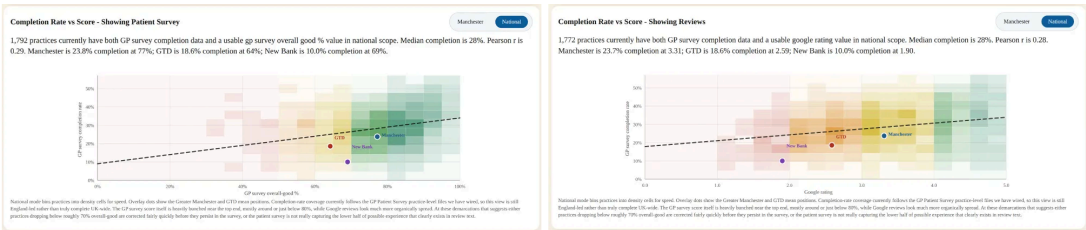
Nationally, I'm not sure if take-away here is the patient survey has no bottom contingent at all (still looking at completion rates, I think they drop out of the data), and not much middle either, but crucially, higher deprivation has worse reviews relative to the patient survey.



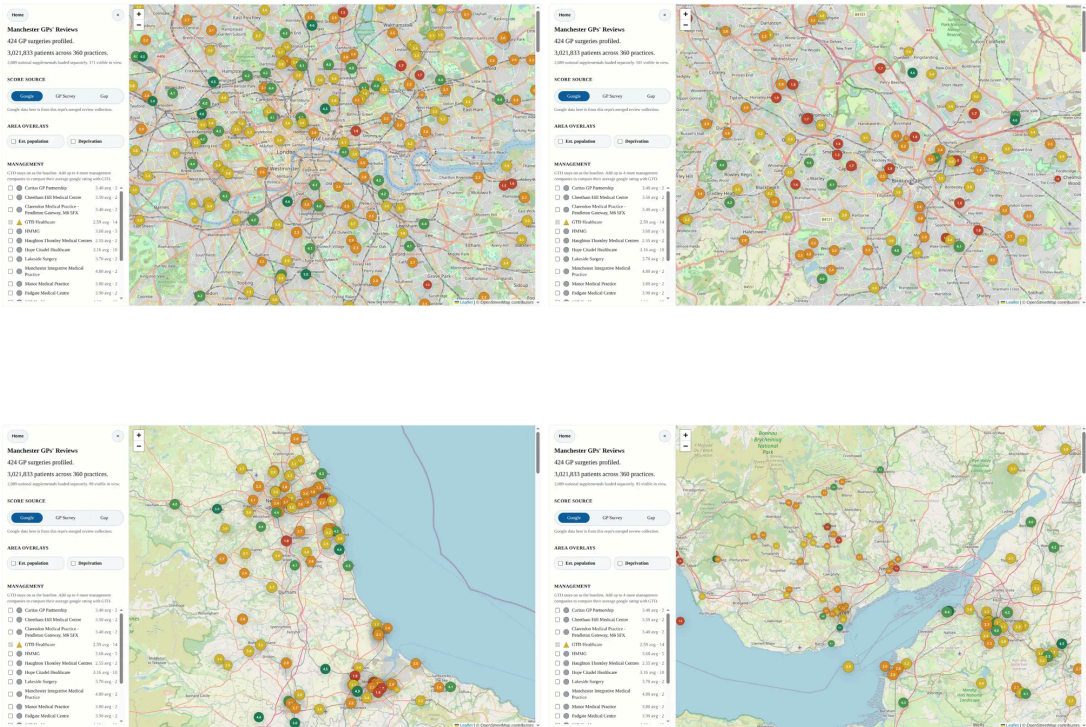
The slightly good news is that patients do leave poor quality GP practices (or at least do join them less slowly, given strong overall pop growth), but it's very slight/slow movement, suggesting the vast majority of patients just stay with crappy/failing practices, despite ~50% access complaints



Perhaps unsurprising that nationally and in Manchester, low-reviewed practices see generally lower patient-survey response rates. The patient survey itself shows this even stronger.

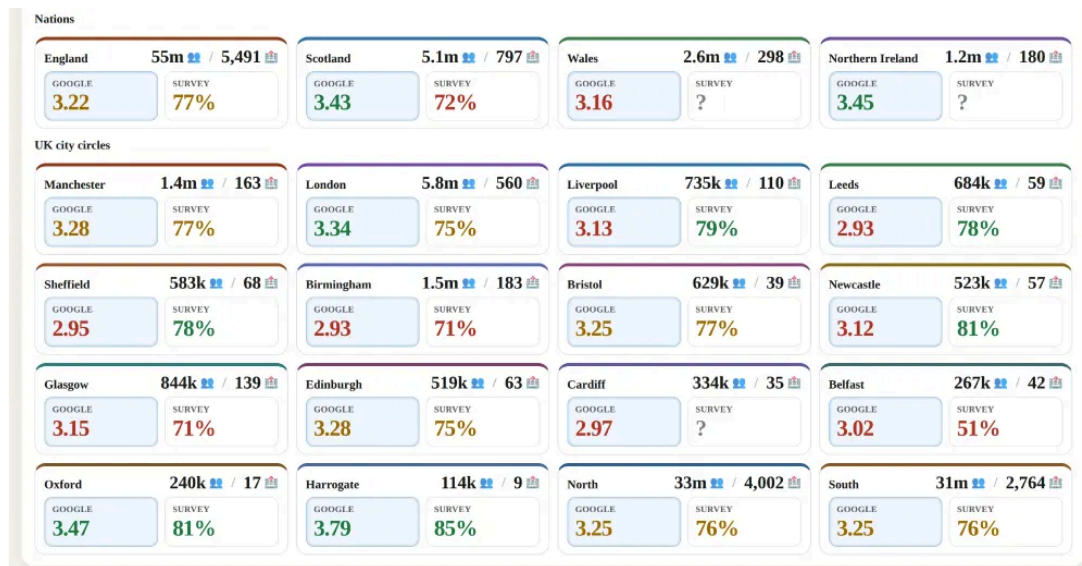


London, Birmingham, Newcastle, Cardiff. Doesn't seem to be much of a clear north-south split, but comparing say Bristol to Cardiff there's clearly a distinction there.

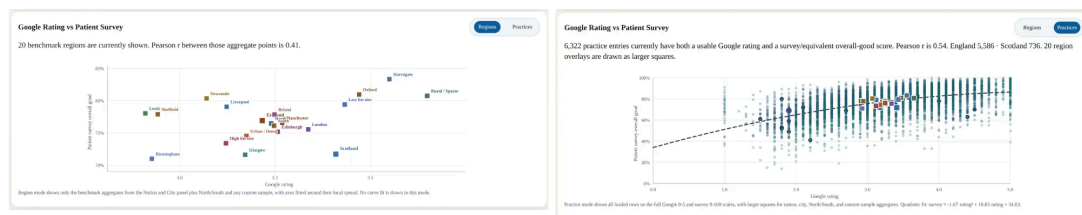


Want to be guaranteed easy access to a good doctor? Move to Stratford, Hampstead, West Yorkshire, Littleborough, Morpeth, Southport, Lincoln's pretty good, and most of the Scottish islands/coast.

Was a little surprised to find no north-south divide, and the spread between rich and poor definitely isn't as large as it could have been, but it's still a little too clear that poorer places have worse GP reviews.



The curve is pretty close to a straight line (good), suggesting the survey _does_ capture meaning (wider than it is tall), despite the broad spread and the survey's much narrower expression. Practices very rarely survey below 60% but this suggests it's cos most practices aren't quite _that_ bad.



Still a lot of work to do on this, but I added a GP finder that uses catchment areas and suggests the best doctors you can register for (England only). Think I'll port it to its own tool on BBB, without the reports stuff (or to accompany it). bobdavies.co.uk/nhs-complain...

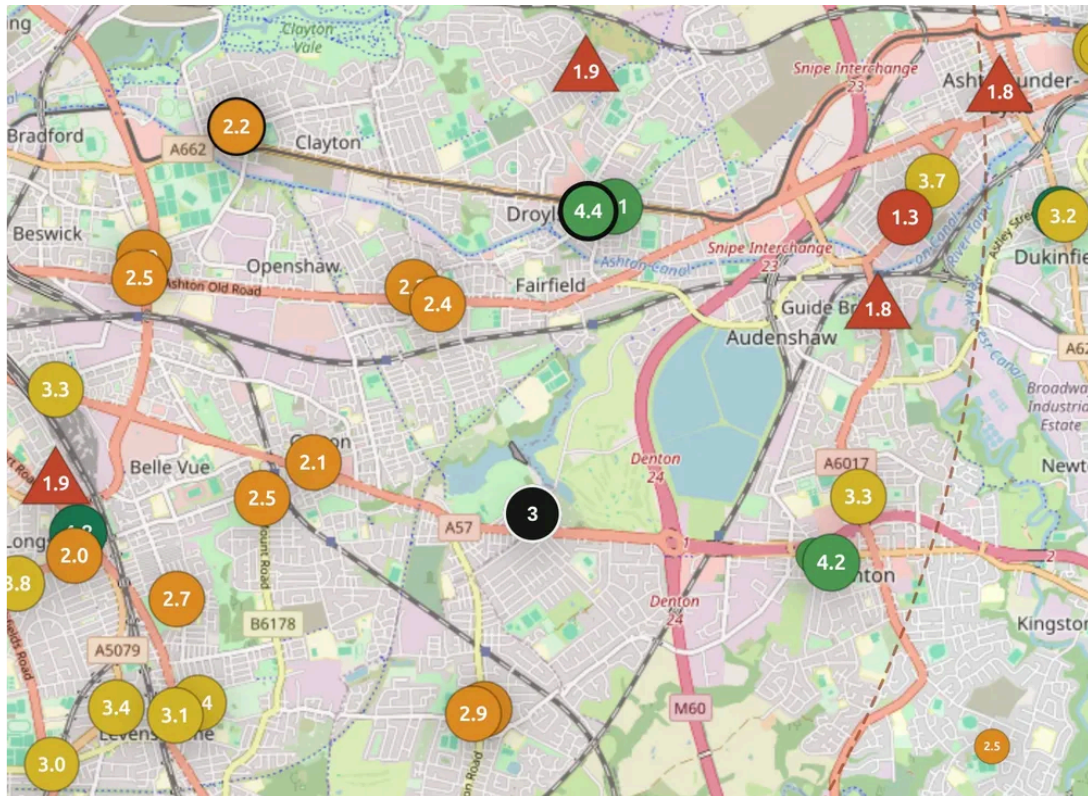
TIL: NHS practice catchments are often insane. Haven't found out how they decide these yet, but it seems to be mostly up to each practice. I didn't find many big voids, but there are some healthcare deserts where the only available practices have poor reviews, even with a good practice nearby.

I had a bit of a poke around and didn't find any examples of 'good' practices' catchments obviously skirting deprived areas, but it'd be a pretty easy way to bump your numbers as a practice manager, so might have to test more thoroughly for it against the IoD data.

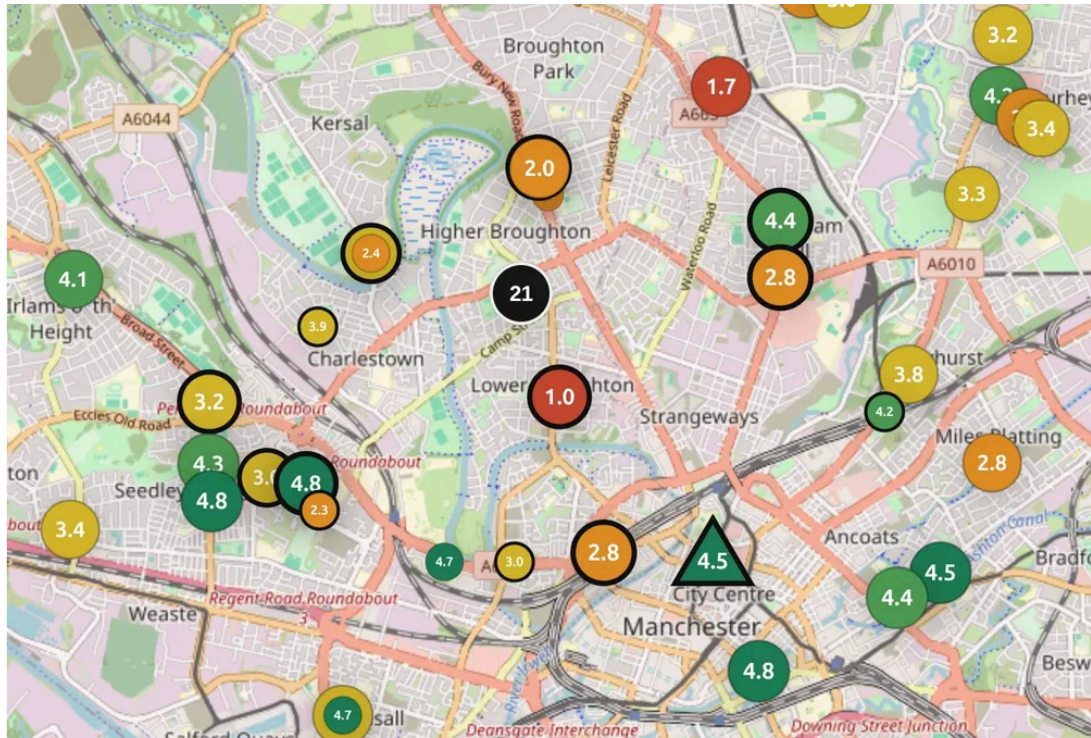
I found a lot of bad/poor practices choosing very small catchments (like my own doctor) which covers barely a single neighbourhood block. I'd love to know what causes

that kind of scale-down, cos it's not universal, some bad practices in comfortably covered areas still set massive catchments.

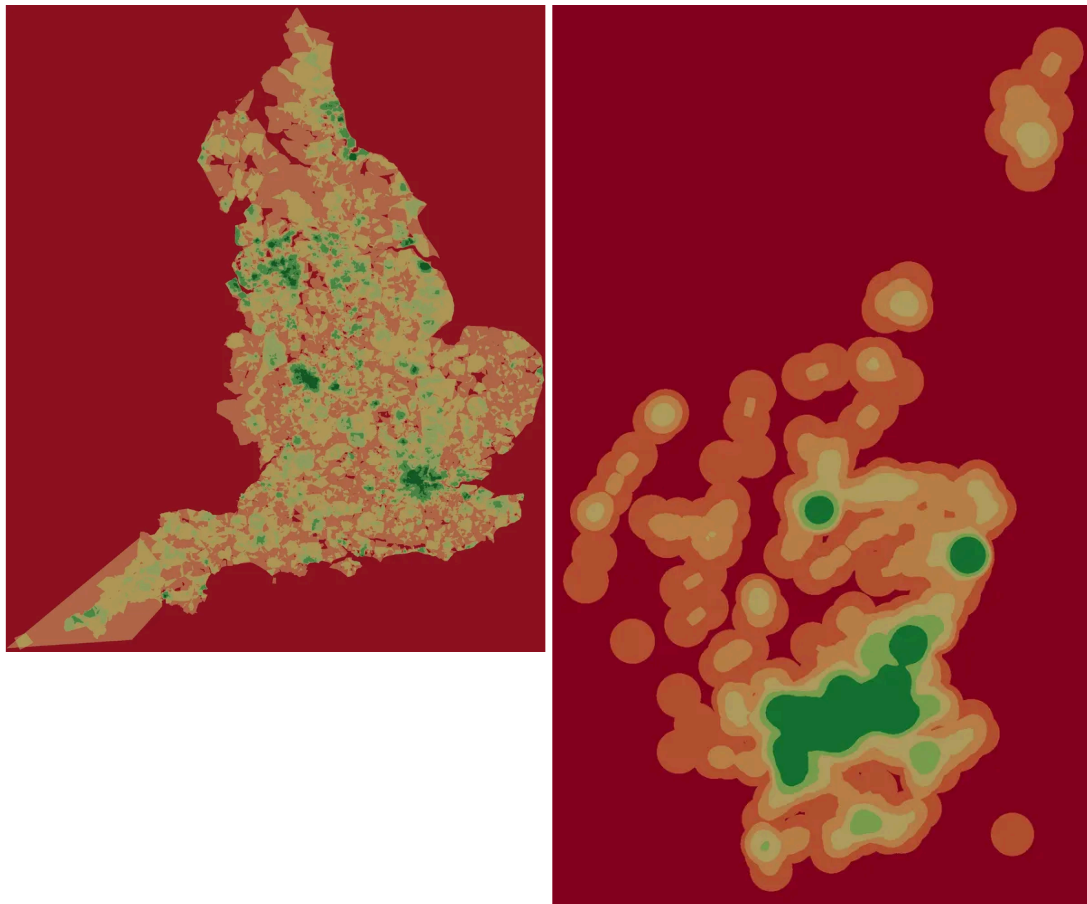
Little spots like this are far too easy to find. Dozens of nearby doctors, but only in the catchments of 3 much further away, because practices just draw lines along roads, with no planning/coordination.

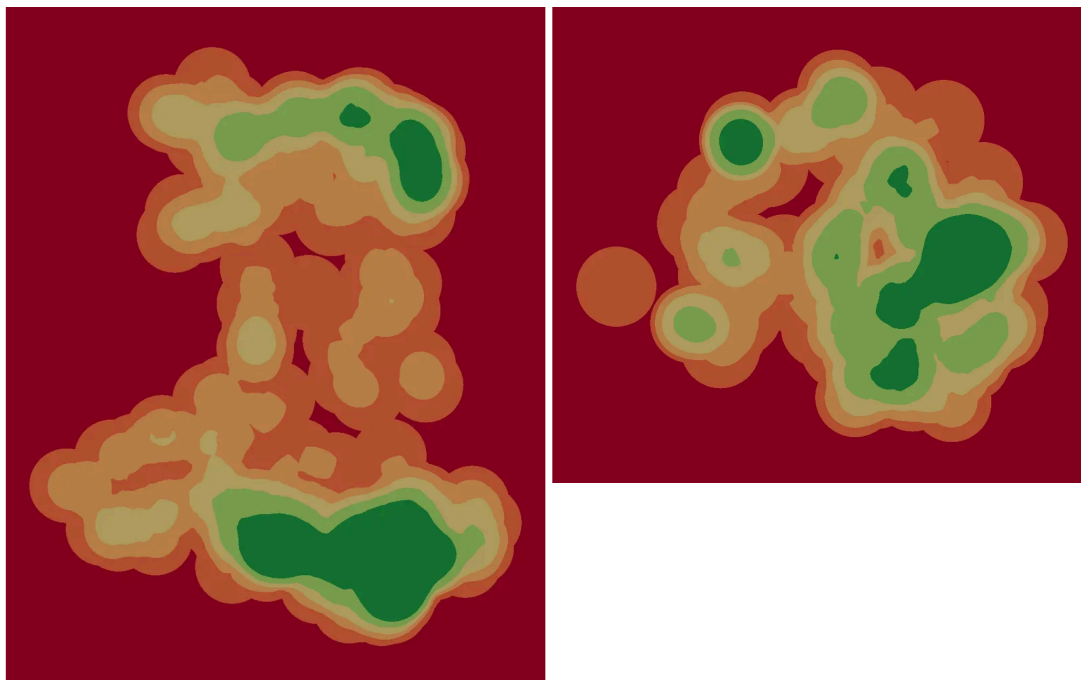


Compared to this, from scanning around (I think calculating intersections is probably going to be worth it) in dense/urban areas, a pool of around 8-13 GPs is normal, in light/rural areas, around 3-5 within reasonable travel times, but a lot of poorer areas are stuck choosing quality or distance.

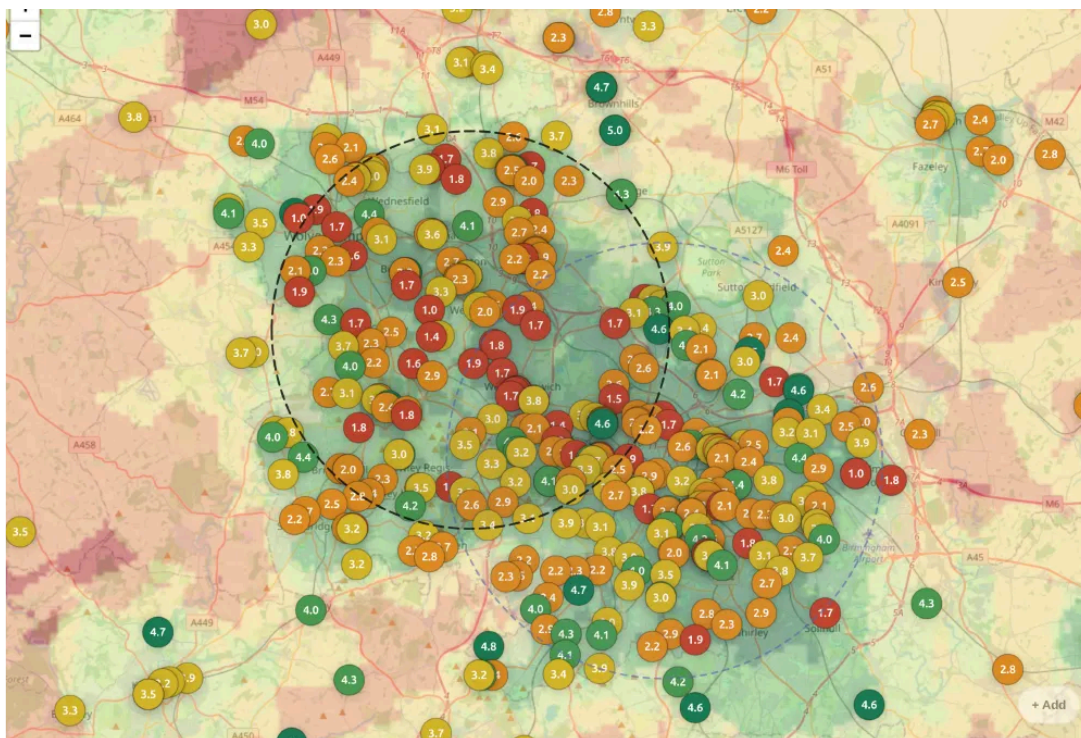


All 4 nations' GP practices coverage (England has exact catchment data, Wales, Scotland and Northern Ireland are simple range circles. Some pretty big gaps, but you can pretty much see they're where you'd expect (mostly the middle of nowhere) but there are some cooler spots in poorer/smaller cities.





Going by the patient survey, there's a roughly 50:50 chance of picking a good practice randomly (the one you just live by, doing no research), but the chance of living near a practice with good reviews on Google is closer to 1 in 5. Between Birmingham and Wolverhampton is probably the worst.



There's a pretty clear east-west divide in coverage (the east coast will have you travel further to a doctor), but they do tend to have higher ratings (rural/sparse)

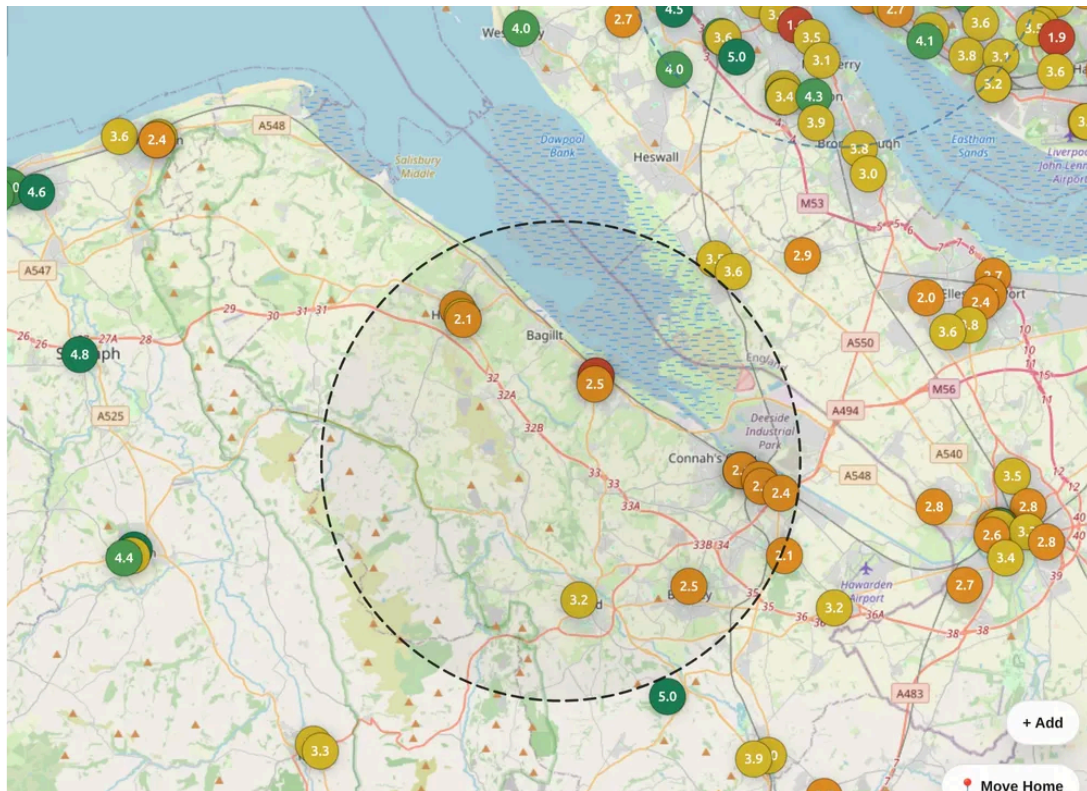


Wales, Scotland and Northern Ireland have slightly large/generous catchment ranges (since those NHS orgs don't maintain/publish catchments, only NHS England, which seems a little odd given how crucial they can be), so more data will probably shrink 'real' coverage in those nations.

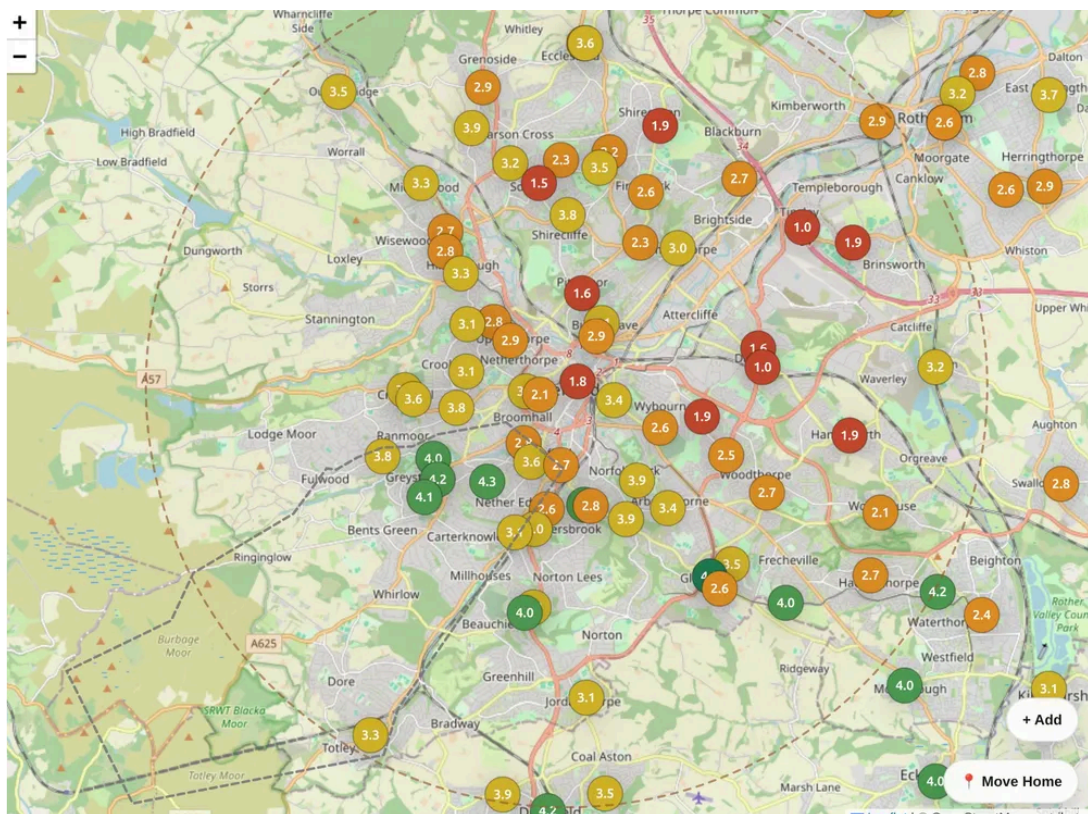
I've improved performance at all zoom levels by having practices render near your cursor, and the practice picker works well now, so you can find the best practice anywhere. There's really 2 websites smashed into 1 here, so they'll be split soon I think.

bobdavies.co.uk/nhs-complain...

This is what it looks like when there's just no good choices. Then people blame them for being thick, for voting against the system that gives them barely the dregs.



Similarly in Sheffield. Every single one of those green practices has a tiny catchment that barely covers itself and a few neighbourhoods, while the rest of the city gets trash-to-middling.



Part of what I keep seeing in how catchments are often laid out, is some (usually well rated) practices drawing quite careful lines around specific (usually relatively wealthy) neighbourhoods, and then a bunch of struggling GPs with overlapping catchments dealing with the poorest areas.

Seems like a failure of the weird semi-free-market ideals (like make businesses bid for the work, and then having a complicated 'analysis' phase that picks the lowest bidder with a really long complicated reason) but early stages of contracts research looks like the crappy areas get many fewer bids.

The handful of management companies actually bidding for the toughest jobs are usually the ones that are obviously struggling financially, obviously failing patients on accessibility, and clearly can't afford to hire good enough staff (and relaxed enough process) to fix their real problems.

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